DBT Program Fidelity Self-Evaluation

© 2026 White Crane Training Collective. All rights reserved.

1. DBT Policy and Procedures Manual

Agency: _____ **Reviewer:** _____ **Date:** _____

CRITERIA

The agency DBT program manual has been approved by the administration of the agency and clearly communicates the philosophy of DBT consistent with the evidence-based model. The manual meets the following criteria:

- ☐ DBT Policy and Procedures Manual is written according to DMH criteria provided.
- ☐ The manual has been reviewed and signed as approved by administration.
- ☐ Manual has been updated with any program changes at least annually.

SCORE = 1	SCORE = 3	SCORE = 5
The manual is present but incomplete and other criteria are not met. ☐ 1. DBT Policy and Procedures Manual is missing components/information (see separate checklist for this item). ☐ 2. Manual has not been approved and/or signed by administration/QA. ☐ 3. Manual has not been updated in the last year and/or does not reflect current program functioning.	Manual is complete but one or two of the other criteria are missing.	All criteria are met.

SELECTED SCORE (1–5)

☐ **1** ☐ **2** ☐ **3** ☐ **4** ☐ **5**

Source of information: Review of Program Manual

Rater Notes:

2. Program Mission and Philosophy

Agency: _____ **Reviewer:** _____ **Date:** _____

CRITERIA

The program has a clearly published mission consistent with DBT that is:

☐1. Articulated clearly in the policies and procedures manual.

☐2. Appears on printed materials (brochures, informational handouts to clients, etc.).

☐3. Is known by key staff in the agency (all team members, QA personnel, Administrators).

SCORE = 1	SCORE = 3	SCORE = 5
☐1. Mission is not present in manual or other print materials or is not consistent with the DBT philosophy.	Mission and philosophy are present in the policies and procedures manual but some key staff are unable to articulate it.	Mission and philosophy are present in the policies and procedures manual and any other print materials where it might be expected (brochures, program data reports, client orientation handouts, etc.) and can be clearly articulated by key staff members when questioned.

SELECTED SCORE (1–5)	
☐ **1** ☐ **2** ☐ **3** ☐ **4** ☐ **5**	**Source of information:** Review of program manual and printed materials (brochures, handouts); interview questions with key staff.

Rater Notes:

3. Admission Criteria

Agency: _____ **Reviewer:** _____ **Date:** _____

CRITERIA

☐Inclusionary/Exclusionary criteria for the DBT program are documented in the manual and known to all intake personnel.
☐A screening tool and/or referral form has been identified and made available to all intake staff.
☐The screening tool is used with all clients who enter the agency for the purpose of counseling services.
☐Referrals are clearly based upon the outcome of the screening tool.

SCORE = 1	SCORE = 3	SCORE = 5
☐1. Inclusion and Exclusion criteria are not clearly defined by the program in the DBT Policy and Procedures Manual or otherwise. ☐2. No specific screening tool has been identified for use in screening clients. ☐3. The screening tool is not used with all clients at intake. ☐4. Referrals are not based upon the outcome of the screening tool.	Inclusion/Exclusion criteria are established and are roughly adhered to by intake staff, but there is no specific screening tool identified and/or used for referral.	All criteria are met.

SELECTED SCORE (1–5)
☐ **1** ☐ **2** ☐ **3** ☐ **4** ☐ **5**

Source of information: Interview questions, example screening tools, chart reviews, team survey, QA data

Rater Notes:

4. Individual Therapy

Agency: _____ **Reviewer:** _____ **Date:** _____

CRITERIA

- ☐ An individual therapist is assigned to each client and is responsible for modifications of the treatment plan.
- ☐ Therapy is held weekly for 50-60 minutes, with frequency adjusted as clinically appropriate.
- ☐ The structure of a session is followed with most aligned interventions (check-in, diary card review, setting agenda, chain analysis, intervention, homework assignment, and wind-down).
- ☐ Target behaviors are addressed in the order of the treatment hierarchy (1. Life-Threatening Behaviors; 2. Therapy-Interfering Behaviors; 3. Quality-of-Life-Interfering Behaviors; and skills teaching).
- ☐ The four primary DBT interventions are applied as applicable in session: skills training, exposure, cognitive modification, and/or contingency management, as appropriate to the treatment plan and goals of the client.

SCORE = 1	SCORE = 2	SCORE = 3	SCORE = 4	SCORE = 5
No individual DBT therapist is assigned. Therapist is assigned but shows no evidence of delivering DBT treatment to the client as evidenced by at least 5 of the following: ☐ 1. Therapist meets less than weekly with client for first 6 months and/or less than every other week thereafter. ☐ 2. No or inconsistent requirement/use of diary card. ☐ 3. No or inconsistent evidence of use of the hierarchy of targets to guide session focus. ☐ 4. No or inconsistent assessment of target behaviors (via suicide assessment and/or behavior chain analysis). ☐ 5. No or inconsistent use of solution analysis. ☐ 6. No or inconsistent evidence of the use of primary interventions in DBT. ☐ 7. No/inconsistent use of or follow up on session homework from week to week.	Clients are assigned a DBT therapist but documentation shows limited evidence of delivering DBT treatment to clients as evidenced by at least 4 of the items on the checklist in the left column.	Clients are assigned a DBT therapist but documentation shows limited evidence of delivering DBT treatment to clients as evidenced by at least 3 of the items on the checklist in the left column.	All previous criteria are met. Therapist does not make use of the full range of DBT interventions but leans heavily on one or two interventions and no mention is made in the record of the use of other interventions.	All prior criteria are met. In addition, the therapist documentation shows that they actively "pull" new behavior out of client by getting them to try new behaviors in session, assign highly relevant homework, and follow up on assignments from week to week.

SELECTED SCORE (1–5)

☐ **1** ☐ **2** ☐ **3** ☐ **4** ☐ **5**

Source of information: Interview questions, therapy notes, DBT consultation notes.

Rater Notes:

5. Group Skills Training

Agency: _____ **Reviewer:** _____ **Date:** _____

CRITERIA

☐1. Stage One DBT clients participate in skills training, delivered in a group format or (more rarely) in an individual format separate from the individual session.

☐2. If conducted in a group format, two skills leaders are present for at least 80% of full sessions.

☐3. A skills training group consists of a weekly 2-2.5 hour classroom-type setting, with up to eight members per group when at 3/4 census or more.

☐4. Every effort is made to ensure Stage One DBT clients attend group skills training.

☐5. In rare circumstances (e.g., due to medical or psychological symptom severity or lack of an available group), clients may receive skills training individually; this is reviewed with the team.

SCORE = 1	SCORE = 2	SCORE = 3	SCORE = 4	SCORE = 5
No skills training is scheduled, skills training is held but is not scheduled weekly and/or is canceled frequently. Skills training departs significantly from fidelity to the DBT model as evidenced by at least 6 of the following issues: ☐1. If skills are taught in a group format, two co-leaders are not present for full groups at least 80% of the time. ☐2. The DBT Skills Training Manual (Linehan 2015 or 2025) is not used as a source for skills training and/or content is incorrectly taught. ☐3. The structure of skills training is not followed (mindfulness, homework review, break, teaching). ☐4. Skills training is less than 60 minutes (individual sessions, in home sessions), 90 minutes (groups of three or fewer clients) or 120 minutes (groups of more than 3 clients). ☐5. Each individual in skills training is not required to report on their observations of mindfulness and/or homework and/or pertinent questions aren't asked to ascertain client understanding. ☐6. Exercises are not done in group to pull new behaviors from clients as they learn skills. ☐7. Group is dismissed early or canceled more than two times per year. ☐8. Homework is not required and/or does not meet the following criteria: instructions are clear, assignments are relevant to the topic/client needs.	Skills training is being held on a regular basis but departs somewhat significantly from fidelity to the DBT model as evidenced by at least 5 of the issues in checklist in left column.	Skills training is being held on a regular basis but departs from the fidelity to the DBT model as evidenced by at least 4 of the issues in checklist in left column.	Skills training is being held on a regular basis but departs from the fidelity to the DBT model as evidenced by at least 2 of the issues in checklist in left column.	The skills training program either meets full criteria previously noted or had two or fewer of the issues on their rating form.

SELECTED SCORE (1–5)

☐ **1** ☐ **2** ☐ **3** ☐ **4** ☐ **5**

Source of information: interview questions, up-to-date group schedule, team survey, progress notes that reflect which skills or page of the manual is being worked on

Rater Notes:

6. Skills Coaching/Crisis Intervention

Agency: _____ **Reviewer:** _____ **Date:** _____

CRITERIA

Clients are able to access their individual therapist between individual therapy sessions without arbitrary limits on time of day. The purpose of these calls is to learn adaptive ways to ask for help and enhance skills generalization between groups and sessions, to strengthen skills, and to avoid the occurrence of target behaviors. In addition, phone availability offers an avenue for clients to repair relationships following conflict or misunderstandings without having to wait until the following session. Coaching contacts (by phone, text, or in person) are brief and not for the purpose of conducting therapy or conducting full crisis assessments and/or interventions. These unscheduled contacts are not to be conducted within 24 hours of a suicide or self-harm behavior, though the client may contact the therapist for help in getting medical attention if needed.

SCORE = 1	SCORE = 2	SCORE = 3	SCORE = 4	SCORE = 5
Coaching calls are not available to clients by their individual therapists at all or are consistently not available for arbitrary portions of any 24 hour period.	Coaching calls by the individual therapist are available to all clients 24/7, however, there are significant departures from the DBT model as evidenced by 5 or more of the following issues: ☐ 1. Therapists do not orient clients to the uses, processes, and limits of therapist availability at the beginning of treatment and in an ongoing manner (e.g., I'm sick/will be on vacation, etc., and will be unavailable and another therapist will be taking coaching calls). ☐ 2. Clients do not call for coaching and the therapist does not assess whether or not this is a therapy interfering behavior (TIB) and/or does not treat the TIB if defined as such. ☐ 3. Therapist does not document that they are following the structure and including critical elements of a coaching call. ☐ 4. Therapist takes coaching calls without assessing whether or not self-harm/suicide behavior occurred in the 24 hour period prior to the call. ☐ 5. Therapist knowingly coaches clients within 24 hours of self-harm/suicide behaviors (does not apply to adolescent clients). ☐ 6. Therapist engages in "heart-to-heart" conversations with the client, making calls excessively reinforcing. ☐ 7. Therapists do not document any follow-up plans or lack thereof in the record of calls.	Coaching calls by the individual therapist are available to all clients 24/7, however, there are significant departures from the DBT model as evidenced by 3 or more of the issues on the checklist in the SCORE = 2 column.	Coaching calls by the individual therapist are available to all clients 24/7, however, there are departures from the DBT model as evidenced by 2 or more of the issues on the checklist in the SCORE = 2 column.	Coaching calls by the individual therapist are available to all clients 24/7, and there are no more than one of the issues on the checklist in the SCORE = 2 column present in the coaching element of the program.

SELECTED SCORE (1–5)

☐ **1** ☐ **2** ☐ **3** ☐ **4** ☐ **5**

Source of information: Interview questions, coaching call documentation; meeting notes (if available), consultation team notes

Rater Notes:

7. Consultation Team Meetings Occurrence/Functions

Agency: _____ **Reviewer:** _____ **Date:** _____

CRITERIA

Weekly consultation team meetings are necessary to improve the skills and morale of DBT providers. These are scheduled and attended weekly by the entire identified consultation team, including non-agency counselors listed as part of the team. Focus is therapy for the therapist model. Care is taken to follow a schedule of topics in team, minutes are taken, and minutes are saved for team/administrative reference and for review by the fidelity team. The purpose of the consultation team and meetings is to help team clinicians identify and address problems arising in the delivery of their treatment. It serves as a forum to ensure clinicians do not fall into iatrogenic behaviors, and if they do, that these issues are addressed quickly. The focus is on treating the therapist, less on case consultation, using DBT strategies.

SCORE = 1	SCORE = 2	SCORE = 3	SCORE = 4	SCORE = 5
Consultation meetings do not occur on a weekly basis. Team minutes must be recorded consistently to document this item. Canceled team meetings and the reasons for not meeting should be documented.	Consultation team meetings occur on a weekly basis but there is significant departure from the DBT model as evidenced by the presence of 5 or more of the following issues: ☐1. Team meetings are canceled more than 4 times a year (counting for holidays, etc.) and/or attendance is not tracked consistently. ☐2. Team members do not consistently (at least 90% of the time) attend consultation team meetings. ☐3. There are no contingencies established for being late, leaving early, or missing team meetings. Or the contingencies are present but are not followed. ☐4. Team does not consistently follow a schedule of events in the meeting, does not track time for each item and/or runs out of time to complete the agenda. ☐5. Team roles are not planned/followed. ☐6. The majority of the meeting focuses on problem solving client issues (and not therapy for the therapist). ☐7. Administrative issues absorb more than 15 minutes of meeting time. ☐8. The team does not have a plan of training and/or does not use any team time for DBT training. ☐9. Team minutes are not kept consistently or are not adequate to provide recall of structure/process/topics.	Consultation team meetings occur on a weekly basis but there is significant departure from the DBT model as evidenced by the presence of 4 or more of the issues on the checklist in the SCORE = 2 column.	Consultation team meetings occur on a weekly basis but there is significant departure from the DBT model as evidenced by the presence of 3 or more of the issues on the checklist in the SCORE = 2 column.	Consultation team meetings occur on a weekly basis with departure from the DBT model as evidenced by the presence of 2 or fewer of the issues on the checklist in the SCORE = 2 column.

SELECTED SCORE (1–5)

☐ **1** ☐ **2** ☐ **3** ☐ **4** ☐ **5**

Source of information: Interview questions, consultation meeting notes, review of DBT policies and procedures manual

Rater Notes:

8. Life-Threatening Behaviors Assessment and Intervention

Agency: _____ **Reviewer:** _____ **Date:** _____

CRITERIA

Suicidal, self harm, and homicide behaviors are assessed at the outset of treatment and regularly throughout treatment during individual sessions and coaching calls. The record must show:

- ☐1. Formal Initial assessment for suicide, self-harm, and homicide (by history and current).
- ☐2. Releases of information to a minimum of 1 emergency contact in the community. Contacts are kept up to date and available to the therapist.
- ☐3. A crisis management plan that includes relapse prevention strategies.
- ☐4. Documentation and utilization of a full risk assessment of suicide/self-harm behavior when indicated.

SCORE = 1	SCORE = 2	SCORE = 3	SCORE = 4	SCORE = 5
Team is treating clients without adequate risk assessment/intervention as indicated by at least 4 of the following issues: ☐1. No clear documentation exists that the therapist (intake or ongoing) completed an initial, formal risk assessment. ☐2. There is no documentation of regular monitoring for suicide/self harm/homicidal behaviors. ☐3. There are no updated releases of information for emergency contacts. ☐4. There is no clear documentation of a crisis/relapse prevention plan with specific strategies for the client to use. ☐5. There are instances of suicide/self-harm/homicidal thoughts, urges, and/or behaviors in the record that were not addressed with a risk assessment.	Team is treating clients without adequate risk assessment/intervention as indicated by at least 2 of the issues on the checklist in the left column.	Team is treating clients without adequate risk assessment/intervention as indicated by at least 1 of the issues on the checklist in the left column.	Team is generally treating clients with adequate risk assessment/intervention, with issues in previous rating levels met. There remain issues with at least one of the following: 4. Crisis/safety/relapse prevention plans are not updated regularly (e.g., every three months at a minimum or as needed). 5. There are instances of suicide/self-harm/homicidal thoughts, urges, and/or behaviors in the record that were not addressed with a risk assessment and no explanation is provided for this gap.	Risk assessment, intervention and documentation thereof are implemented as expected with none of the issues in previous ratings present.

SELECTED SCORE (1–5)

☐ **1** ☐ **2** ☐ **3** ☐ **4** ☐ **5**

Source of information: Interview questions, contact info documentation, progress notes, non-billable notes, assessments in patient records.

Rater Notes:

9. Ancillary Treatments/Care Providers

Agency: _____ **Reviewer:** _____ **Date:** _____

CRITERIA

On occasion, clients may be involved in treatments/care in addition to DBT. The DBT therapist, in careful consultation with the DBT team, will determine if ancillary treatments are appropriate and indicated. Examples of these ancillary treatments are: pharmacotherapy, day treatment, employment programming, AA or other self-help groups, case management, specialized eating disorder treatment, ACT, ITCD, IPS or other outside providers. With these ancillary treatments, the consultant-to-the-client approach will usually be employed. The DBT therapist will coach the client on interacting and communicating with other members of the team or care providers. The individual therapist will communicate with non-DBT providers independent of the client only as necessary. The following criteria must be met:

- ☐ 1. When the client is in comprehensive DBT, the team has final decision making ability in collaboration with the client as to whether a client engages in ancillary treatment. This is documented in the DBT Policies and Procedures Manual.
- ☐ 2. If an ancillary treatment is considered, the team discusses this before making a decision considering the possible benefits and costs (e.g., treatment fatigue, problematic contingency management) for the client.
- ☐ 3. The client is included in any discussions/decisions about ancillary treatment prior to taking action.
- ☐ 4. The client is coached in communication with ancillary providers.
- ☐ 5. Team members only interact with ancillary providers with the permission (releases of information) of the client and most often in the presence of the client.

SCORE = 1	SCORE = 2	SCORE = 3	SCORE = 4	SCORE = 5
If team is not the final decision-maker in the client's assignment to ancillary treatments, this should be noted in the record, each time such a decision is made and the client begins an ancillary treatment. If the team demonstrates at least 4 of the following issues, they are practicing substantially outside the guidelines for a DBT provider: ☐ 1. There exists no policy for the team regarding ancillary treatment. ☐ 2. Referrals for ancillary treatment occur without consult with the team. ☐ 3. The client is referred without effective discussion about their own motivation to do the ancillary treatment, the purpose of the treatment from the team's perspective, etc. ☐ 4. The individual therapist does not check in about ancillary treatments during individual sessions and/or does not coach the client in managing interactions, dealing with their own reactions in that treatment, or other implications of the ancillary treatment for the client. ☐ 5. Team members interact with the ancillary provider primarily independently of the client.	If the team demonstrates at least 3 of the issues on the checklist in the left column, they are practicing substantially outside the guidelines for a DBT provider.	If the team demonstrates at least 2 of the issues on the checklist in the left column, they are practicing outside the guidelines for a DBT provider.	If the team demonstrates any one of the issues on the checklist in the left column, they are practicing outside the guidelines for a DBT provider.	Team meets all prior criteria.

SELECTED SCORE (1–5)

☐ **1** ☐ **2** ☐ **3** ☐ **4** ☐ **5**

Source of information: Interview questions, releases of information, progress notes, informational notes, collaborative contact notes. 1) Team assesses the need for or for existing ancillary treatment 2) includes them in discussion and treatment planning 3) assists and guides the client with ancillary treatment 4) The DBT therapist collaborates with other existing treatment/care providers

Rater Notes: *If the agency administration does not support the DBT provider as decision-maker for clients, this should not result in a scoring penalty during the first two rating periods for the team, but rather is included in a summary of expectations in the report provided to administration. Following the second such occurrence, scoring for the team is impacted.*

10. Training

Agency: _____ **Reviewer:** _____ **Date:** _____

CRITERIA

All new program staff** receive standardized training in DBT prior to beginning their involvement on the team or during the first six months of their work with the team. For those providers who do not receive all elements of the training prior to joining a team, they should be individually supervised until training is completed. These standard trainings may be taken in Missouri or from other trainers but must include:

- ☐ 1. Introductory topics (research on effectiveness, overview of therapeutic processes (e.g., dialectics and validation), each component of the comprehensive treatment (individual, skills, coaching, and team), primary treatment interventions (cognitive modification, exposure strategies, skills, and contingency management), and review of skills modules).
- ☐ 2. A DBT-based individual therapy course that teaches the structure (included activities, schedule of a session) and processes (goal setting, treatment planning, communication strategies, behavior chain analysis, solution analysis, and balancing acceptance and change).
- ☐ 3. A suicide assessment and intervention course that addresses long and short-term risk assessment, suicide and self-harm intervention from a DBT perspective, crisis management, and suicide/self-harm treatment management.
- ☐ 4. The program has a training plan in place that is updated annually, and has documentation of ongoing training and support of DBT providers on the team. This can include recurrent reviews in team of relevant materials (suicide assessment and intervention, advanced strategies in DBT, etc.). In addition to the above standard initial trainings during the first year, staff are required to complete 10 hours annually in DBT-related in-services or trainings for all DBT staff. These hours of training may be completed in-team, via team retreats, and/or in readings completed, but records of such training should include the timeframe, title, description, objectives, and (for trainings providing a certificate) the signature of individual providing the training.
- ☐ 5. New team members must receive a 1-2 hour introduction to consultation team processes and structure, which can be conducted by a trainer or by a member of the team. This should be documented. All trainings must be provided by trainers who have been approved by the DMH under their criterion for qualified DBT training providers. **Staff is defined as all individuals providing DBT and includes consultation team members.

SCORE = 1	SCORE = 2	SCORE = 3	SCORE = 4	SCORE = 5
Team is operating outside the fidelity expectations of DBT as evidenced by at least 5 of the following: ☐ 1. There is no documented training policy in the treatment manual. ☐ 2. There are no records of initial or ongoing training completed by members of the consultation team. ☐ 3. There is no evidence of an updated annual training plan for the team. ☐ 4. There is no indication that new members have been asked about training or have been oriented to the consultation team by someone when they began work on the team. ☐ 5. Some team members beyond the 6 month point of joining team have not received one or more of the standard trainings in DBT. ☐ 6. Trainings have been taken/documented for workshops not led by approved trainers/agencies.	Team is operating outside the fidelity expectations of DBT as evidenced by at least 4 of the items on the checklist in the left column.	Team is operating outside the fidelity expectations of DBT as evidenced by 3 of the items on the checklist in the left column.	Team is operating outside the fidelity expectations of DBT as evidenced by 2 of the items on the checklist in the left column.	All criteria for training are met by the team with the exception of one or fewer of the items on the checklist in the left column.

SELECTED SCORE (1–5)

☐ **1** ☐ **2** ☐ **3** ☐ **4** ☐ **5**

Source of information: Team survey, interview questions, DBT Policies and Procedures Manual

Rater Notes:

11. Termination and Transfer Procedures

Agency: _____ **Reviewer:** _____ **Date:** _____

CRITERIA

The program identifies termination procedures in the DBT Policies and Procedures Manual and there is evidence in the documentation of individual sessions and/or consultation team meetings that the team follows these practices as described. The policy and practices should include at a minimum:

- ☐ 1. During the orientation and commitment phase of therapy, termination of therapy is discussed (expected end date, goals to have been accomplished, what happens after termination, etc.)
- ☐ 2. During active treatment every effort is made to increase the independence of the client in seeking needed assistance outside of treatment.
- ☐ 3. Active discussions about termination with the client begin approximately 2/3 to 3/4 through the expected length of treatment. Tapering of sessions may also begin at this time as indicated by acuity and other factors.
- ☐ 4. The role of "ex-therapist" and "ex-client" should be thoroughly discussed so the client understands what to expect from the therapist following the conclusion of treatment (p. 459 in Linehan, 1993; treatment is terminated, but not the relationship between therapist and client).
- ☐ 5. Therapists discuss any problems clients terminating from treatment may be likely to expect immediately following termination. Problem-solving and skills practice are used to reduce the likelihood of these problems. Problem solving may include booster sessions as needed after termination.
- ☐ 6. Evaluation of progress: Time should be made to review progress to date, gains the client would like to continue to make, and processing how the relationship between therapist and client has grown, changed, and impacted both individuals.
- ☐ 7. Make appropriate referrals. If acuity allows, taking a break from therapy after graduating from DBT can be useful. If the client has additional goals to continue to work on beyond DBT then appropriate referral to other providers is made (substance use treatment, trauma work, etc.).

SCORE = 1	SCORE = 2	SCORE = 3	SCORE = 4	SCORE = 5
The team does not follow the DBT treatment manual regarding therapeutic procedures of termination as evidenced by at least 6 of the following issues: ☐ 1. There are no policies or procedures related to termination in the agency's DBT treatment manual. ☐ 2. Records of clients who have completed treatment do not show that termination was discussed during orientation and commitment. ☐ 3. No explicit mention is made in individual therapy notes about increasing the client's autonomy in managing crisis behaviors or engaging in social support. ☐ 4. There is no evidence in the record of any active discussions about termination prior to one month before termination. Sessions were not tapered and no explanation provided. ☐ 5. Relationship and contact expectations between therapist and client were not discussed prior to termination and/or the therapist indicated that no further contact is possible. ☐ 6. No discussion about possible issues following termination was held, no problem solving occurred. ☐ 7. There was limited/no discussion of the progress the client made in treatment, their ongoing goals and how to pursue these, or processing the relationship between therapist and client. ☐ 8. Referrals were not made—either left open or not documented.	The team does not follow the DBT treatment manual regarding therapeutic procedures of termination as evidenced by at least 5 of the issues on the checklist in the left column.	The team does not follow the DBT treatment manual regarding therapeutic procedures of termination as evidenced by at least 4 of the issues on the checklist in the left column.	The team shows significant departures from the DBT treatment manual regarding therapeutic procedures of termination as evidenced by at least 3 of the issues on the checklist in the left column.	The team follows the DBT treatment manual regarding termination well, as evidenced by having 2 or fewer of the issues on the checklist in the left column.

SELECTED SCORE (1–5)

☐ **1** ☐ **2** ☐ **3** ☐ **4** ☐ **5**

Source of information: Interview questions, discharge charting information, discharge policies

Rater Notes:

12. Quality Monitoring/Improvement Processes

Agency: _____ **Reviewer:** _____ **Date:** _____

CRITERIA

Program leaders and administrators collect process data including, but are not limited to: internal fidelity measures, client diagnoses, number of clients admitted, number of discharges, and number of dropouts. Data is aggregated and used to drive changes in the program to increase fidelity to the model or quality of the services provided.

SCORE = 1	SCORE = 2	SCORE = 3	SCORE = 4	SCORE = 5
No evidence that formal process monitoring occurs	Informal process monitoring is used or informal monitoring is provided less than annually	Formal process monitoring is completed annually with no analyzing of data to drive program improvements	Formal process monitoring is completed annually AND data is analyzed to drive program improvements	Evidence that formal process monitoring is completed: 1) more than once annually, AND 2) the data are analyzed and drive program improvements.

SELECTED SCORE (1–5)

☐ **1** ☐ **2** ☐ **3** ☐ **4** ☐ **5**

Source of information: Interview questions, Team survey, QA policies, QA outcomes reports, QA meeting notes or minutes, dashboard indicators, program QA data reports

Rater Notes:

13. Quality Monitoring/Client Outcomes

Agency: _____ **Reviewer:** _____ **Date:** _____

CRITERIA

Program leaders and administrators monitor outcomes for DBT consumers every 3 months and share data with DBT practitioners**. Monitoring involves a standardized approach to assessing key outcomes related to DBT, for example, psychiatric admissions, self-harming incidents, incarceration, substance use treatment scale, or employment rate. QA processes can be a joint effort between counselors and leadership. **DBT practitioners include those providing direct DBT services and members of the consultation team.

SCORE = 1	SCORE = 2	SCORE = 3	SCORE = 4	SCORE = 5
No evidence that formal outcome monitoring completed	Outcome monitoring completed at least 1 time a year, but results are not shared with the DBT practitioners	Standardized outcome monitoring completed at least 1 time a year. Results are shared with DBT practitioners	Standardized outcome monitoring completed at least 2 times a year. Results are shared with DBT practitioners	Standardized outcome monitoring 1) Completed quarterly AND 2) Results are shared with DBT practitioners

SELECTED SCORE (1–5)

☐ **1** ☐ **2** ☐ **3** ☐ **4** ☐ **5**

Source of information: Interview questions, team survey, QA policies, QA outcomes reports, QA meeting notes or minutes, dashboard indicators, program QA data reports

Rater Notes:

Score Summary

Agency: _____ **Reviewer:** _____ **Date:** _____

Category	Score (1–5)	Notes
1. DBT Policy and Procedures Manual		
2. Program Mission and Philosophy		
3. Admission Criteria		
4. Individual Therapy		
5. Group Skills Training		
6. Skills Coaching/Crisis Intervention		
7. Consultation Team Meetings Occurrence/Functions		
8. Safety Assessment		
9. Ancillary Treatments/Care Providers		
10. Training		
11. Termination and Transfer Procedures		
12. Quality Monitoring/Improvement Processes		
13. Quality Monitoring/Client Outcomes		
TOTAL SCORE		
Average score (Total ÷ 13)		

Note: 13 criteria, maximum total score of 65. Cutoffs are scaled proportionally from the original 14-criterion scale (62/70 and 52/70).

Implementation level (Total Score)

☐58–65 = Good implementation

☐48–57 = Fair implementation

☐47 and below = Not evidence-based practice